



Consultation
Nicolas

RAT RIVER HEALTH COUNCIL

**A Vision for the future of the
Centre médico-social DeSalaberry District Health Centre**

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A Vision for the future of the Centre médico-social DeSalaberry District Health Centre

1.0 Background

For many years the communities in the Chaboillé region (which includes the rural communities of Dufrost, La Rochelle, Otterburne, Saint-Malo and Saint-Pierre) have been passionately involved in ensuring the renewal of health services in their region. The construction of the new building for the Centre médico-social DeSalaberry Health District (the Centre) was the ultimate in demonstrating the results of community mobilization.

Thereafter, when discussions regarding regionalisation abounded, the communities demonstrated a renewed interest in the delivery of health services in their district. As a result, in 1994 the communities positioned themselves to take an active part in health reform by creating the Rat River Health Council (RRHC). Members of the Council were and continue to be representatives primarily from the Village of Saint-Pierre and the Municipality of DeSalaberry. The RRHC became the vehicle for community mobilization and education. To this day the RRHC, “a voluntary grass root action committee” continues to play a lead role in matters relating to health and health care.

By the time South Eastman Health/Santé Sud-Est (SEH) was created in 1997, the communities in the Chaboillé region had a grasp of regionalisation and were able to contribute and influence the decision making processes. Shortly after its inception, South Eastman Health created four health advisory councils which were representative of the four planning districts i.e. the Central, Southern, Western and Northern health advisory councils.

The health advisory councils, representing the voice of the communities, were initiated for health planning purposes and they act in an advisory capacity to the Board of Directors of South Eastman Health. For the Chaboillé region, the western district health advisory council provided another avenue for ongoing input in the regionalisation process and the strategic planning for the region.

The desire to be involved, to be full participants in the decision making processes, to be consulted and informed as the regional health authority moves forward in renewing the health system is inherent to the culture of the Chaboillé region. This keen interest continues to be the driving force behind any and all involvement by the communities. This environment creates an ideal opportunity for creativity and innovation in planning stable and sustainable health services in the western district.

2.0 Introduction

Outstanding concerns related to physician recruitment and retention, the quality of health services delivery, the stability and sustainability of the programs and services provided at the Centre médico-social DeSalaberry (The Centre) are ever present. In the last two years, the communities began to actively explore different avenues to resolve those issues.

In 2004, the communities embarked upon three key strategies to deal with the growing concerns noted above, including:

- a study was conducted to identify the effects of regionalisation of health services on the Sainte-Anne and Saint-Pierre-Jolys communities ¹;
- a community survey relating to medical services in the St. Pierre & DeSalaberry area was conducted ²;
- a workshop on a S.W.O.T. analysis for the DeSalaberry District Health Centre: doctor recruitment and retention was held in April 2004 ³.

The outcomes of the three key strategies further underlined the legitimacy of the community concerns and emphasized the need to take further action. As a result of the workshop conducted in April 2004, the Coalition was created to act as a political lobby group. Membership on the Coalition is comprised of representatives from the business community in the Chaboillé region.

The Coalition actively pursued South Eastman Health and Manitoba Health for effective resolution of the outstanding concerns, most importantly, the growing physician crisis in Saint-Pierre and the perceived diminishing health services in the district. Two initiatives were addressed:

- South Eastman Health, under the direction of the Director of Medical Affairs conducted a review of the medical clinic in Saint-Pierre ⁴;
- in the fall of 2005, the RRHC and the Chaboillé Coalition contracted the services of Suzanne Nicolas of Consultation Nicolas to provide direction and assist the communities in the effective resolution of issues.

¹ PJR ACTION, (Octobre 2004). Effets de la régionalisation des soins de santé sur les communautés de Ste Anne et de St-Pierre-Jolys. Étude réalisée pour le compte de L'Association des municipalités bilingues du Manitoba.

² Rat River Health Council, (November 2004). Survey of medical services in the St. Pierre & DeSalaberry Area.

³ Jim Jacq & associates, (April 2004). Building blueprints to success: DeSalaberry District Centre S.W.O.T: Doctor recruitment and retention planning session.

⁴ Dr. Ferd Pauls, (August 2004). St. Pierre Medical Clinic Review.

2.1 Consultant's mandate

To effectively deal with the complexities of the health system, the concerns of the key stakeholders and the effective resolution of issues, the consultant pursued a three pronged approach:

1. review of information and statistical data on health programs and services offered at The Centre; this will provide an opportunity to better inform the community and to ensure a common understanding by all key stakeholders;
2. conduct a staff survey to gain some insight into staff's opinions and perspectives of programs and services, the Centre's management and operations, quality of services provided and staff needs;
3. review existing reports e.g. the community health assessment 2003/04, the community survey on medical services in the Saint-Pierre and DeSalaberry area and other pertinent literature.

2.1.1 Anticipated outcomes

The consultant will work through a systematic and transparent process to maximize collaboration between the communities, South Eastman Health and other key stakeholders. The outcome of this process is to recommend strategies which will lead to the articulation of a clear vision for the future of the Centre médico-social DeSalaberry District Health Centre. A number of options will be explored:

1. the expansion or reorganisation of existing services;
2. the addition of new programs and services if deemed necessary;
3. the provision of an integrated, coordinated and accessible continuum of health services;
4. the provision of quality services in the language of choice.

2.1.2 Methodology

A small working group was formed consisting of 3 representatives from the Coalition and the RRHC to assist and guide the consultant through the various steps of the process. The working group was consulted when necessary and were kept informed of the progress.

Once the project was underway, key stakeholder groups including South Eastman Health, Saint-Malo and Saint-Pierre Chamber of Commerce, the Town Council, the DeSalaberry Municipal Council, the physicians and staff of the Centre were informed of this initiative.

South Eastman Health agreed to fully participate in the project by allowing the consultant full access to all necessary documentation and staff. The communities agreed to await the outcomes of this process prior to initiating any other course of action.

A review of all programs and services offered at the DeSalaberry Health Centre was carried out. The review included the following programs:

- Acute care services, including palliative care, the Emergency Room (ER), laboratory and diagnostic services;
- Personal care home (Repos Jolys);
- Community palliative care program;
- Community services including Public Health, Home Care, Mental Health;
- Physiotherapy Program;
- Emergency medical services (EMS);
- Coin santé (Health Corner);
- Saint-Pierre medical clinic;
- Dental services;
- Midwifery program;

A number of methods were used to obtain information on the programs and services. These consisted of:

- interviews
 - one on one interviews with:
 - program managers and/or delegates (10 staff);
 - staff of various departments at the Centre (20 staff);
 - physicians and staff at the medical clinic (6);
 - South Eastman Health senior staff
- 2 focus groups with physicians;
- review and analysis of statistical information of programs and services;
- review of the South Eastman Health website, brochures and pamphlets.

An important first step in the project was to illicit staff participation via the completion of a staff survey. The survey was conducted to gain some insight into staff's opinions and perspectives of programs and services, the Centre's management and operations, the quality of services provided and staff needs. In addition, staff was able to identify how their employer could be of support to them.

The staff survey was developed in collaboration with senior staff at the region. The survey was further validated by the working group. South Eastman Health assumed responsibility for the distribution of the staff surveys (a French and English version) were distributed with the pay stubs the week of October 7th, 2005. A total of 99 surveys were distributed to all staff housed at the Centre. Staff had the opportunity to complete the survey in the language of choice i.e. French or English. Staff was given a three week period to complete the survey and was asked to return the survey through a return postage paid envelope.

A third component of the project was to review documents which could provide insight into the health planning process for the future of the Centre. These included:

- the 2003/04 South Eastman Community Health Assessment;
- the 2004 community survey of Medical Services;
- South Eastman Health strategic plan (2006-2011);
- South Eastman health plan (2006-2007);
- other relevant literature review.

The first draft of the report and its recommendations was validated by the RRHC, and the Coalition. The content of the report was also shared with South Eastman Health prior to printing of the final report.

3.0 Staff survey

Surveys were distributed to ninety-nine staff at the Centre. Of these, 32 questionnaires were completed and returned and 20 employees were interviewed, for a total response rate of almost 52%. The duration of the structured interviews ranged from 45 minutes to 2.5 hours.

The questionnaire was divided into four sections: The Centre and its operations, client/patient/customer services, staff needs and programs and services (Appendix A). Staff had the opportunity to add any additional comments as well as being able to have a direct interview if they so desired.

Data was analyzed using a systematic interpretive approach. The field notes, the interviews and the completed questionnaires provided the text for analysis and interpretation. All data was read several times to arrive at the general themes. The findings will be presented according to these themes.

3.1 Survey findings: What is the staff saying?

3.1.1 What staff appreciates ...

The facility and its services:

Overwhelmingly, staff at the Centre stated that having the health care facility located in the community was greatly appreciated. Having the diversity of services “under one roof” is an asset to the staff, as it offers an opportunity to develop a varied skill set. The dedication and commitment of the staff and volunteers was noted. Staff is described as being caring, friendly and helpful. The recent appointment of the facility manager is described as being a great asset to the facility.

Quality of services provided:

Staff is proud of the services they provide. Eighty per cent (80%) of staff qualify the quality of the services provided as being good to very good, but the quality can vary depending on the circumstances. Staff also indicated that the ability to provide bilingual services (French and English) improves the quality of services provided at The Centre.

Quality of work life

It is interesting to note that although staff is faced with a challenging work environment, the majority of staff (86%) still describes the quality of their work life as being good to very good. Staff report that they enjoy their work and are proud to be working at the Centre.

3.1.2 What staff least appreciate ...

Lack of communication:

The lack of communication was identified as a key factor when staff described what they appreciate the least about the Centre. They feel they are uninformed about the future of the facility and therefore worry about whether there will be a facility closure or if in fact, the hospital will be transformed into a personal care home. They describe not being informed and not understanding the many changes that continue to occur at the Centre (for example, the most recent change related to the scheduling protocol). Staff feels that “no one seems to care about what happens to them”.

Many staff members are feeling they are “overworked, understaffed and are frustrated”. Low staff morale abounds. The “lack of communication between staff and departments within the facility”, “the cut backs”, “downsizing”, “skeletal resources”, “staff doing their own thing”, “unsafe care”, all contribute to a sense of staff not feeling valued. This in turn is creating a sense of apathy amongst staff which can sometimes lead to “poor quality of care”, “lack of continuity and consistency in the care provided” and most importantly, giving the impression that “the client/customer is an inconvenience to staff” or “patients are a burden”.

Reporting structure:

Staff are clear that they are accountable to their immediate supervisors i.e. the Clinical Resource Nurse (CRN) or the Regional Manager however, there is a great deal of confusion surrounding how that reporting mechanism is operationalized on a practical, day to day basis. For example, support staff report to a regional director that is located outside of the facility and “is not easily accessible”. When issues arise and the regional manager is not accessible, does staff access the Facility Manager for resolution of issues? If so how are issues dealt with between the Facility Manager and the Regional Manager? Who is ultimately responsible for the decision making process?

A further example relates to the role and responsibilities of the CRN in the acute care facility. The health care team does not understand the nature of the authority in that role.

Lack of accessibility to services:

Two other areas of concern emerged i.e. the closure of the emergency department and the instability of physician services. In as far as the emergency department, there is a sense of hope that the department will reopen once “things settle down with physicians”. As for the instability of physician services, staff’s main focus was around the many issues related to the lack of accessibility to the medical clinic and the low morale of the physicians.

3.1.3 What would staff change?

Create an environment that values its staff, residents and families

Activities would include:

- “communication, communication, communication” with:
 - the patients, residents, families,
 - within the facility itself (between staff and departments, with physicians, supervisors, facility manager);
 - with the community;
 - with South Eastman Health.
- creating a newsletter to inform staff of change;
- improve morale: increase staff appreciation, listen to staff, display a caring attitude, treat staff equally and with respect, be staff advocates;
- provide access to an employee assistance program;
- pay equalization;

Create an environment that promotes the delivery of quality services

Activities would include:

- have a clear accountability structure;
- have clear job descriptions, follow standards and protocols, regional policies;
- encourage team work;
- increase staff in-services in the respective departments e.g. maintenance of critical skills, ER triaging and nurse managed care, building maintenance, occupational health and safety, etc.
- review work schedules, rotations, workload distribution in the various departments;
- review workload and need for increased resources;
- provide updated equipment;

Create an environment where services are readily available and accessible

- retention of physicians for improved accessibility and continuity of care;
- reopen the emergency room 24/7;
- more accessible laboratory services;
- improve operations of the medical clinic
 - more accessible services;
 - decrease wait times for appointments;
 - provide walk in services;
 - give priority to staff who are ill and need to be seen during the workday;
 - provide training to reception staff on “how to treat clients”;
 - better working conditions for staff;
 - add other professionals to core staff e.g. nurse or nurse practitioner;
 - more bilingual staff

Other comments

- more effective use of existing space;
- spend less time on surveys and more time on improving working conditions.

3.1.4 Staff's views on programs and services

Staff comments were solicited to gain insight on the perception of the future role of The Centre. As well, staff was encouraged to identify what other programs and services they felt were required to better meet the needs of the community.

Many of the staff members stated that “it is not a question of adding new health services at The Centre, but rather needing to improve and build upon the existing services”. A number of suggestions were raised for consideration, for example: minor surgery, midwifery, adult day hospital, hospice care, ultrasound, optometrist, regional palliative care program and a chemotherapy program, improve on call services for laboratory and diagnostic services.

3.1.5 Other comments

Concerns raised by a few staff in the facility related to a number of other issues:

- expectations of nursing staff are considerable given that on shift and weekend there are only 2 nurses covering the entire facility:
 - they have to perform ward clerk activities;
 - they have to perform phlebotomies on shift when necessary;
 - they have to give medications at the Repos Jolys;
 - they have to perform nurse managed care in the emergency department;
 - they have minimal support staff to assist in the nursing activities.
- lack of support by the community:
 - community members not “doctoring” in Saint-Pierre;
 - community not accessing their emergency care at The Centre;
- staff recruitment and retention
 - more difficult to recruit professional staff due to the bilingual requirement;
 - if physicians resided in Saint-Pierre, could potentially build more stability in the delivery of medical services;
- there is a need for community education related to:
 - the use of the emergency department;
 - improving the public image of The Centre and its services.

4.0 What is the community saying?

The community is expressing growing concerns about the stability and the quality of the existing services at The Centre. It is interesting to note that the community has the same concern as the staff, that is, the fear that The Centre will close or that it will be transformed into a long term facility. Stakeholders face numerous challenges, for example:

- the lack of stability of the Saint-Pierre medical clinic, the rapid turn over of physicians, lack of continuity of care;
- lack of accessible primary care services;
- the lack of bilingual health services;
- onerous on call schedules for physicians;
- difficulty in recruitment of bilingual staff, including physicians;
- the recent closure of the emergency department from 2000 – 0800 hours;
- articulating a clear vision for the future of the DeSalaberry District Health Centre.

Amidst all of these challenges, the communities have clearly and resoundingly articulated the need to:

- provide health services which are coordinated and integrated across the continuum of care;
- provide accessible, client focused services;
- provide quality services which includes services in the language of choice (French or English);
- review and reorganize existing services and potentially expand services to better meet the needs of the community;
- develop stable and sustainable health services at The Centre;
- clearly articulate a vision for the future of The Centre.

4.1 Review of community survey

In 2004, the RRHC conducted a survey regarding medical services in the Saint-Pierre and DeSalaberry Area. The survey focused on the Saint-Pierre medical clinic and the emergency room at The Centre. The survey was distributed to individuals residing in the rural communities of Dufrost, Otterburne, Saint-Malo and Saint-Pierre. Two thousand, one hundred and thirty-four (2,134) surveys were distributed. 756 individuals responded to the survey for a respond rate of 35.4%.

The characteristics of the respondents are as follows:

- 60% were female;
- 92% were over 31 years of age;
- all resided in the Saint-Pierre area and/or the DeSalaberry Municipality (51 respondents did not identify a place of residence);
- the average # of years lived in the area was 34 years;

- 58.7% of respondents receive their primary care services in Saint-Pierre, 41% receive services elsewhere including: 11.9% in Steinbach, 18.4% in Winnipeg, 3.3% in Saint-Anne, 1.6% in Saint-Malo, 1.1% in Niverville and 5% elsewhere;
- 255 respondents indicated that the medical clinic they were using was not their first choice, of these 157 or 62% would wish to receive services in Saint-Pierre.

The respondents who do not access their primary health services in Saint-Pierre (157 respondents) would decide to do so if the following circumstances were present:

- ✓ if they had access to physician of choice (94%);
- ✓ if there were shorter waiting times for appointments (94%);
- ✓ if there was assurance of continuity of care by same doctor (96%);
- ✓ if bilingual services were available (53%);
- ✓ other (20%).

For individuals who decided not to receive there services in Saint-Pierre (314), they were asked the reasons for their decision. This is how they responded:

- 83% indicated they were not able to get an appointment;
- 98% indicated they would have an extended waiting time for their appointment;
- 76% said the physician of choice was not accepting appointments;
- 89% indicated that the physician turnover was too great;
- 27% indicated they could not get services in both official language;
- 45% had an unpleasant experience;
- 20% indicated the clinic requested that their medical files be transferred;
- 14% indicated other reasons.

Two questions in the survey related to the quality of services at the Saint-Pierre medical clinic. One question dealt with the quality of services when calling the clinic, of which 68% of respondents rated as satisfactory or excellent. The other question addressed the quality of services received when visiting the clinic; 75% rated those services as satisfactory or excellent.

There was only one question asked regarding expectations of the hours of operations for the emergency room at The Centre. It is not surprising that an overwhelming 91% of respondents indicated their preference for hours of operation to be 24 hours/day, 7 days/week.

5.0 The future of health services at the DeSalaberry District Health Centre

Key stakeholders including the communities of the Chaboillé region and South Eastman are presently at a crossroad. They face numerous challenges that require attention if the implementation of a stable and sustainable health delivery system is to be achieved at The Centre. Thus far, since the inception of regionalization, perpetual administrative and operational changes have occurred at The Centre. At this time, more attention must be directed towards the planning of future health services. Critical issues requiring attention includes:

- human resources availability and stability (critical situation with physicians);
- the stability and sustainability of the medical clinic
- the use of the acute care facility (e.g. under utilization of hospital beds, low patient volumes);
- the use of the emergency department and its intermittent closures;
- minimum staffing levels
- on-call rotations for physicians;
- the waiting list at the Repos Jolys;
- demographic changes within the region;

This section of the report attempts to establish the foundation upon which to plan for the future of health services at The Centre. The environmental scan will provide the basis upon which to make the recommendations.

5.1 Environmental scan

Future health planning for The Centre must occur within the broader context of South Eastman Health. The demographic profile of the region must be taken into account as it will have major impact on resources and service utilization. The environmental scan integrates findings from the 2003/04 South Eastman Comprehensive Community Health Assessment, the South Eastman Health strategic plan (2006-2011), the South Eastman health plan (2006-2007) and other relevant information in the region regarding its programs and services.

5.1.1 Demographics

- Between 1998-2004, the South Eastman population increased 11.9%, the Western District (includes the Chaboillé region) experienced a 2.9% population growth.
- Population growth has occurred across all age groups.
- The South Eastman population is somewhat younger than the general Manitoba population with a greater percentage of children under age 15 (24% vs. 20%) and a smaller percentage of seniors age 65+ (11% vs.14%).
- In South Eastman, the senior population is increasing more rapidly than in the rest of Manitoba. Between 1998-2004, the population age 65+ rose 8.2% (compared to

- 1.6% across Manitoba). Vulnerable seniors age 75 and over are increasing at a rate of 13.1%, nearly double the provincial average.
- Seniors are high users of physician and hospital services.
 - Seniors represent 80% of home care clients and over 90% of personal care home residents.
 - Since 1998, South Eastman has experienced a 12% reduction in the number of PCH beds available to the population age 75+; this is reflective of population growth, not closure of beds.
 - South Eastman has the highest proportion of Francophone residents in Manitoba – 14% of South Eastman residents use the French language at home, compared to only 4% in Manitoba. In the Western and Northern districts, 31% and 22% of residents respectively use French language at home.

5.1.2 Centre médico-social DeSalaberry District Health Centre

The Centre médico-social DeSalaberry Health Centre (The Centre) serves as a district health centre and as such houses the hospital, the personal care home (Repos Jolys), the community health services offices, the physicians' clinic, the dental office, the emergency medical services (EMS), the Health Corner and other primary health services. For the purpose of this report, the programs and services provided at The Centre are divided into three major categories:

1. Primary health care services which include:

- Coin santé/Health corner
- Palliative care
- Physiotherapy
- Community services
 - Public Health
 - Mental Health
 - Home Care
- Midwifery Program (offices in Saint-Pierre Jolys)
- Emergency Medical Services (EMS)
- Dental clinic
- Medical clinic

2. Hospital services which includes:

- Acute care
 - Palliative care
 - Alternate care
- Emergency room services
- Laboratory and diagnostic services
- Palliative care

3. Personal Care Home (Repos Jolys)

How will we reach the goal of developing a stable and sustainable health delivery system at The Centre?

The Canadian Institute for Health Information (CIHI) describes the concept of “bringing together health and health services”⁵. Just as there are a number of issues or factors that can be associated with the health problems of an individual, interventions to deal with those health problems are provided in many different “settings” such as private clinics, primary health centres, hospitals, rehabilitation units, personal care homes, ER, in the home or elsewhere in the community. It clearly defines the necessity for a continuum of integrated services to deal effectively with the health problems of an individual or a given population.

Le “Petit Robert” dictionary defines integration as: “l’établissement d’une interdépendence plus étroite entre les parties” or to “form into a whole, to unite with something else”. Integration is a process and it begins with collaboration and coordination⁶. It consists of the following components:

- **Fragmented services:** separate organizations/programs, confusion, duplication;
- **Cooperation:** sharing of program information;
- **Coordination:** sharing of leadership, planning and decision-making;
- **Collaboration:** equal partners, shared goals and commitment, open communication, community involvement, more effective use of resources;
- **Integration:** common values, holistic, person centered inter-disciplinary approach, flexible, responsive, seamless services, community partnerships.

The implementation of an “Integrated Model of Care” is key to achieving a stable and sustainable health delivery system for The Centre. As described in the section “review of health programs and services”, there is a wealth of existing services being provided in the Western district of the region⁷. Although there is some collaboration between providers, programs for the most part work in isolation of one another.

The existing co-location of the majority of programs and services at The Centre will facilitate the integration process. An integrated model of care for The Centre will:

- provide more access to services by:
 - having access to the right care, at the right time, by the right provider in the right setting (i.e. pre hospital emergency care, primary care, promotion, prevention, ER, acute care, long term care);
 - building more capacity across the system;

⁵ Canadian Institute for Health Information (2003). Health care in Canada.

⁶ Manitoba Family Services and Housing, WRHA, Manitoba Health (July 2003). Winnipeg integrated services initiative: A conceptual framework.

⁷ For planning purposes, South Eastman Health/Santé Sud-Est Inc. is divided into four districts: Western (includes the Village of Saint-Pierre, the RM of DeSalaberry, the Town of Niverville and the RM of Ritchot), Northern, Central and Southern.

- increasing availability of access to French language services;
- increasing providers knowledge about the range of services available to the community;
- improve coordination of services to allow the individual to:
 - move more easily through the programs and services offered on site (for example, having a common reception), services offered elsewhere in the region and outside of the region;
 - have better access to more specialised services;
 - access the different services they need in a timely manner (less waiting time);
- improve the quality of health services:
 - increase client and provider satisfaction
 - provide timely services;
 - provide better continuity of care;
 - provide care that is responsive to the needs of individual, families and the community;
- provide more efficient and effective care:
 - health care providers understand their respective roles and responsibilities;
 - better utilisation of staff time and resources;
 - minimize duplication of services;
 - better communication between the providers;
 - increase potential for staff recruitment and retention;
- results oriented care:
 - improved data collection and analysis to determine what action can be taken to improve program and service delivery (evidence based decision making);
 - accountability to the community and other stakeholders;
 - responsiveness to the community;
 - healthier communities.

The recommendations in this report are therefore formulated within the context of creating an “integrated model of health care” for the Centre. As previously mentioned, to facilitate the presentation of the review of existing programs and services, they have been categorized in three major categories. Recommendation(s) for each category are listed at the end of each major section.

5.1.3 Review of existing programs and services

Primary Health Care Programs and Services

A vast array of primary health care services is provided at The Centre. A brief overview of each program/service is provided.

Coin santé (Health corner)

Le Coin Santé/The Health Corner is a unique community resource centre designed by and for the community. It officially opened in November 1998. It provides health education, wellness promotion and disease and injury prevention in both official languages (French and English). It offers the following resources:

- over 475 brochures categorized under 12 different headings (from April 2002 to April 2004, 10,493 pamphlets and brochures were distributed or accessed directly by the community);
- a book (445 books) and video lending service in both French and English in conjunction with the Jolys Regional Library;
- 2 seasons of programming in both French and English are planned each fall and winter (includes seminars, workshops, exercise classes, health days).

There has been a progressive increase in the number and types of information sessions, exercise programs, support groups and special projects (Breast cancer screening, partnerships with the Diabetes education team, palliative care program, Healthy women, health communities, etc.). Participation in all activities is on the rise and every year new partnerships are created.

The director of the Health Corner reports directly to the RRHC.

Community - palliative care program

The Palliative care program is a regional program coordinated by the palliative care coordinator. The program provides the patient and the family access to a team who can provide support in meeting the physical, mental, emotional and spiritual needs of those dealing with a terminal illness. Palliative care services are provided both in the home and in the institutional settings. Services and support can be provided in the home during the terminal phase of the illness, including dying at home if that is the wish of the individual and the family. The care is provided by existing regional staff in the home through the home care program.

Physiotherapy

A physiotherapy treatment room for residents of the Repos Jolys, inpatients and the general public is located at the Repos Jolys. Services from the Bethesda Hospital are provided on site once per week or four (4) hours per week at The Centre. The

physiotherapist provides approximately one (1) hour per week for inpatient/residents and the other 3 hours are spent providing services to outpatients. Assessments, home exercise program and education are being provided on site. If more extensive treatments are required the client must access the equipment at the Bethesda Hospital since the treatment room at the Repos Jolys is not equipped for specialized treatment.

Services are provided to all age groups from children to the elderly. The majority of clients are elderly. This service is well used by the community and clients usually have a wait time of 1 to 1.5 months.

Dental Services

A dental clinic is housed at the Centre. Approximately 2,000 – 2,500 clients receive their dental services at the clinic.

Community services – Public Health

Public Health in South Eastman offers a vast array of programs and services to the entire region. The Western district benefits from all of those services directly under the umbrella of the Centre. Four overarching programs constitute the public health program with many subprograms and services being provided under each major program.

1. Family health
 - prenatal
 - in 2005, 4 series of classes (2 in Saint-Pierre & 2 in Niverville) were provided in the Western district – mid-February, early May, early September, mid-November
 - early years
 - newborn support
 - baby first
 - super start/bon depart – in Niverville, Otterburne, Saint-Malo, Saint-Adolphe classes are held monthly, minimal attendance in Saint-Pierre
 - growing minds
 - early steps/premiers pas
 - stars of the future
 - child health
 - child health clinics offered once per month in Niverville, St. Adolphe, St. Malo, St. Pierre
 - school programs
 - the public health nurse located at the Centre is responsible for 9 schools in the district

2. Communicable disease control
 - immunization
 - sexually transmitted infections
 - travel health
 - disease information
3. Diabetes and healthy living program
 - services offered to the public and to inpatients one day per month at The Centre and one day per month in Niverville
 - 139 active clients (Western district)

Midwifery Program

South Eastman was one of the first regional health authorities in the province to have midwifery service. The three midwives in the region presently have admitting privileges at Saint-Anne and Bethesda hospitals and they attend home births in the region. The midwifery offices are based in Saint-Pierre.

The physicians in the Saint-Pierre medical centre provide antenatal and post natal care to their clients but the delivery occurs elsewhere in the region or in Winnipeg. In the last 4 years, i.e. from 2001 to 2004 the birth rate in the Western district has remained relatively stable with the majority of the births occurring in Niverville, Île des Chênes and Saint-Adolphe.

Western district	2001	2002	2003	2004	Total
Île des Chênes	34	26	31	32	123
Niverville	34	42	38	44	158
St. Adolphe	18	20	23	20	81
St. Pierre	15	14	20	12	61
St. Malo	16	16	16	16	64
Ste. Agathe	13	12	8	11	44
Otterburne	11	7	11	7	36
Vermette	3	4	9		16
St. Norbert	2	0	2	0	4
Dufrost	0	3	1	1	5
Glenlea	0	1	1	0	2
Grande Pointe	0	0	2	2	4
Crystals springs	0	0	0	0	0
Aubiny	0	1	0	1	2
Total	146	146	162	147	601

There have been 601 births in the last 4 years in the Western district as compared to the Northern district at 769 births and Central District at 1440 births.

Due to the considerable number of births, active discussions have occurred over the last number of years to determine if a birthing centre should be initiated in the Western district and if so, where it should be located. The Centre was considered as a potential option. It had been anticipated that the existing OR theatre could be renovated to accommodate a birthing centre.

In an interview with one of the midwives from the region various options were explored as to how a birthing centre could operate at The Centre:

- model at Sainte-Anne and Bethesda hospital: the nurse at the facility acts as a second attendant to the birth and assumes responsibility for the post partum care two hours post delivery;
- model of a home birth: two midwives attend the birth and one midwife remains with the mother from 2 to 4 hours after the birth, then mother is discharged; the assistance of the nurse may be required to relieve the midwife during the postpartum period;
- free standing birthing centre: the midwife is attended by an attendant (does not need to be a nurse, a few are presently in training but will need to be registered with the College of Midwives; as yet that process has not been established).

In further exploring these options, it appears that at this it is not appropriate to have a birthing room at The Centre. The home birth model would be the most feasible however, there is still a possibility that the RN in the acute care facility would need to provide some relief to the midwife.

Community services - Mental Health services

Community mental health services are available to all citizens of the South Eastman region. The program provides services in all community areas and offices and more specifically, in Saint-Pierre, Niverville and in the schools when needed (Western district).

The program offers services to individuals experiencing emotional and/or psychiatric concerns or living with mental illness. The program assists people through direct counselling, psychiatric consult and referrals to other services as appropriate. The following is a list of services provided by the mental health program:

1. access/intake
2. child and adolescent services
3. adult services
4. intensive case management
5. mental health services for the elderly
6. psychiatric consult
7. mobile crisis service

8. proctor services
9. mental health promotion and prevention
10. housing and residential care (as provided through Eden East, located in Steinbach)

The mental health program continues to experience increased requests for services throughout the region. From 2002/03 to 2003/04, incoming referrals rose 19% for adult services and 16% for child and adolescent services.

Community services - Home Care

Home care is a comprehensive community based program that provides essential support in the home to anyone, regardless of age, who requires health care services or assistance in the activities of daily living. Home care can be provided on a short term basis, for example to provide help following hospital discharge or on a longer term basis to provide an alternative to the long term care system until it is no longer possible for a person to remain safely in their home. Services include:

1. personal care assistance with mobility and personal care;
2. home support: activities such as meals, light housekeeping and laundry;
3. health care: health teaching, counselling, nursing care, physiotherapy and occupational therapy;
4. family relief: short periods of in-home relief for the family and care-giver;
5. respite care: longer periods of relief for family care-giver;
6. supplies and equipment: the provision of supplies and equipment for care in the home;
7. adult day programs: offers social and recreational activities for seniors
8. volunteer services: available to help with other activities that can support the client.

Approximately 80% of home care clients in the region are seniors, with the vast majority being 75 years of age or older. Home care needs continue to increase in the region and the number of cases continues to increase at a rate of approximately 3% per year. In the Niverville office however, there has been a marked increase as a result of the assisted living/supportive housing complex. In this community there has been an increase of approximately 10 to 12% of open cases on Home Care.

Emergency Medical Services (EMS)

Provincial funding is allocated to the 11 regional health authorities in Manitoba. The authorities assume responsibility to plan and implement emergency medical response services in their respective regions. There is a current act and regulations establishing standards for training, vehicles, equipment, services and personnel.

There are 11 ambulances in South Eastman Health, 2 of which are located at the DeSalaberry Health Centre. The primary ambulance is staffed with 8 FTEs, sixteen hours

per day (0800 to 2330), 7days per week. The staffing quota is supplemented by casual staff.

Workload had increased significantly in the Saint-Pierre EMS. In reviewing the EMS volumes comparative report, Saint-Pierre's primary call volumes have increased by an average of 20% for the last nine fiscal years (1995-2004) as compared to the regional average of 6.72%. Inter facility transfers have also increased by an average of 19.5% over the same time frame as compared to 13.5% for the overall regional nine year average.

A great deal has been accomplished in Saint-Pierre in the EMS program. Permanent positions have been gradually added to accommodate the increasing workload, new skill sets have been added and educational support has been provided. The concept of pursuing EMS as a professional career is being marketed and planning is occurring around the notion of improving the integration of EMS in the community services delivery system.

EMS staff located at The Centre have been and continue to be active as part of the care team. Their involvement with the team has varied depending on the needs of the facility and the workload of the EMS personnel. In the past, they have provided clerical support to nursing staff, worked in the ER, drawn blood and they continue to be involved in recertifying (CPR) staff at the Centre.

Saint-Pierre Medical clinic

The Saint-Pierre medical clinic is a privately operated clinic. Four physicians are funded by the regional health authority to provide health services to the community of Saint-Pierre, Saint-Malo and Niverville, the hospital and the personal care home. Recently the practice in Niverville has been discontinued. South Eastman Health has recruited two part time fee for service physicians to provide services to the Niverville community.

In March 2005, the Director of Medical Affairs conducted a management review of the Saint-Pierre medical clinic. His report was submitted to South Eastman Health in the fall of 2005. To avoid duplication of efforts, a review of the clinic was not conducted. For the purpose of this report, the physicians were interviewed individually and two focus groups were held. Additional interviews were held with the staff at the clinic, the Director of Medical Affairs at the region and the consultant who collaborated in the management review. In addition, the report of the Saint-Pierre medical clinic management review was perused.

Recommendations from the medical clinic management review are as follows:

1. That an experienced nurse be hired to be responsible for the receptionists, help in the administration of the Clinic and act as liaison between the Clinic and patients in regards to access. Funding this position could be shared between the Clinic and RHA.
2. That the method of shadow billing be changed to become the responsibility of the RHA so that it could be used with confidence for statistical purposes.

3. Change the generation of the on call rota to reflect the shared responsibility of all physicians.
4. Consider an alternative to physician coverage from midnight to 0800.
5. Have Moto Cross use Bethesda or Winnipeg as the referral ER for trauma patients.
6. Work with the RRHC to decrease the number of Level 4 and 5 patients in the ER.
7. Continue to develop the primary care concept by hiring a .5 EFT physician experienced with nurse practitioners and a full time nurse practitioner. A new class of graduates are available in January 2005 from the University of Manitoba. Dr. XXXX is interested in the .5 EFT.
8. Organize the clinic so that the health care team can provide care to patients in an efficient and timely fashion. This would include, in addition to the above, on a .3 EFT basis Dr. XXXX but no other physicians.
9. That we work closely with the RRHC and other constituents in St. Pierre to re-establish their confidence in the RHA.
10. That we capitalize on the initiative demonstrated by the Niverville community for a primary care model of health care including a nurse practitioner.

In further delving (through the interview process) into the administration and operations of the medical clinic it appears that the entire primary care service delivery system (all services presently being provided at the clinic) need to be optimized within the context of improving access to services. This can include (not an exhaustive list):

- improving how daily activities are carried out and the systems which are used to support the daily operations;
- developing guidelines, protocols – standardizing ways to respond to clients and to give consistent messages, information and advice;
- optimizing the role of all staff i.e. the right person doing the right action at the right time;
- rotating support staff through the various functions to improve efficiencies;
- improving internal communication, cooperation and collaboration in the clinic;
- the team learning to cooperate in new ways;
- considering innovative and creative ways of shaping the demand for visits for example:
 - standardizing appointment types
 - eliminating automatic return visits at standard intervals
 - increasing interval time for return visits
 - interact with client differently
 - optimize client involvement
 - nurse or other allied health professional can manage clients
- providing the necessary support and ongoing continuing education to staff;
- providing all staff with competitive remuneration and benefits.

Considerations for the future of Primary Health Care at DeSalaberry Health Centre

- the instability in the provision of medical services;
- the difficulty in recruitment and retention of professional staff;
- the lack of responsiveness and/or inability to respond to community needs and the changing demographic;
- programs working in isolation of one another or with little collaboration;
- the lack of continuity of care (the information does not follow the client);
- the lack of coordination and integration among programs and services;
- poor access to services (problems with access of after hours services often results in the use of the emergency room for non-urgent care);
- client, community dissatisfaction.

South Eastman strategic plan (2006-2011):

Strategic priority related to primary health care is: “To expand primary health care by modelling all community-based services on primary health care principles, focusing on prevention and health education approaches”.

Objectives:

- to increase the quality and effectiveness of services provided at the first level of contact with the health system;
- to enhance prevention, health promotion, health education and health service integration;
- to meet expressed community needs for greater emphasis on front-line health services, new approaches to service provision, increased accessibility, increased flexibility and alternate modes of delivery.

Outcomes:

- expansion of sites providing integrated service delivery;
- further community outreach bring education, information and wellness activities closer to people’s homes;
- a comprehensive review of potential need for primary health centres in other locations;
- the development of primary health care centres in cooperation with medical centres to reconfigure existing medical services to the primary health care model

Recommendation for the enhancement of Primary Health Care at the DeSalaberry Health Centre:

Given that the vast majority of PHC are presently being provided under “one roof” at The Centre, it is ideally positioned for the establishment of a primary health care centre. This recommendation is multifaceted and includes the following (see diagram of model in Appendix B):

- designate the PHC centre as a bilingual centre i.e. providing services in both French and English;
- ideally, all services at The Centre (Acute care, Repos Jolys, PHC, support services) should be under one leadership i.e. the facility manager;
- establish clear reporting lines between the Coin santé and the facility manager (The Coin santé could also retain an accountability to the RRHC);
- reconfigure the medical clinic into a primary health care model (in collaboration with the Saint-Pierre physicians);
 - clinical leadership will be provided by a medical director amongst the Saint-Pierre physicians;
 - physician contracts will require review, could potentially consider becoming test site for rural Manitoba for new funding model under the “Manitoba: A new approach to primary care”;
- provide a single point of entry to all PHC services;
- improve coordination and integration with existing PHC service (Home Care, Palliative Care, Mental Health, Public Health, Coin Santé/Health Corner, EMS, Midwifery);
- integrate EMS into the PHC service delivery model;
- review the hours of operations for PHC programs and services;
- provide a common information technology infrastructure (e.g. client scheduling system, automated client record, lab order entry and results reporting);
- review potential for sharing of resources (human, financial, material);
- review the space planning program on the basis of the requirements of a PHC;
- create a culture of accountability, performance measurement and quality improvement.

Note: On the basis of the interview with the staff member in the Midwifery program and the review of the options for a birthing centre at The Centre, it is not recommended that a birthing centre be established at The Centre.

5.1.4 The hospital - Acute care services

The hospital is a designated bilingual acute care facility. The hospital services not only the immediate town of Saint-Pierre, but also provides services to the population of the Western district and to out of region patients. The hospital has the capacity and receives funding for 14 acute care beds. It employs seventy (70) full time equivalents (FTEs) and is considered a minimum staffed facility.

The statistical data for the hospital was reviewed in order to identify the types of patients utilizing the facility and to determine how well the beds are being utilized. Data from six fiscal years (1999 to 2005) was analysed and is described in Table 1.

Identifying the types of patients utilizing the hospital beds at the Centre poses significant challenges. Over the years, different methods have been utilized to identify types of patients. It is only in fiscal year 2004/05 that the regional statistics are able to demonstrate with more accuracy the types of patients utilizing the facility.

In 2004/05, 25% of all inpatient days at the Centre were related to alternate care days⁸, no palliative care days were recorded and no cardiac days were counted. It is important to note that the validity of the methods used to gather and collate statistical information has not been reviewed.

Table 1: Total patient days and bed occupancy

	1999- 2000	2000- 2001	2001- 2002	2002- 2003	2003- 2004	2004- 2005
Patient Days	2760	2257	2631	2790	2697	3134
% bed occupancy	54.0	44.2	51.5	54.6	52.7	61.3

For the past 6 fiscal years, the average bed occupancy has been around 53% with 25% of inpatient days related to patients who should not be occupying an acute care inpatient

⁸ Alternate care definition: Patients are categorized as “Alternative care” if they are categorized in one of the following categories:

- patient is waiting to be panelled or awaiting to be place in a personal care home, chronic care facility or companion care placement;
- patient is waiting for home care or community services (home care nursing, home oxygen, home care palliative care, community therapy services, non-nursing home care services, home nutrition services);
- patient is awaiting other placement (foster home, group home, special housing, supportive housing, etc);
- patient in hospital for other reasons for example, respite, homeless, delayed discharge (no one to care for patient, awaiting transport, etc.).

bed. Furthermore, the regional statistical summary for 2003/04 and 2004/05 indicate that the numbers of admission to The Centre has been 305 and 283 respectively, representing less than one admission per day.

The hospital - Palliative Care Program

Palliative care is a program for individuals and families who are facing a life threatening illness that has reached the point where a cure is no longer possible. Palliative care can be provided in the home and is also available in the hospital. If symptoms become unmanageable or if the personal care is too demanding at home, the individual can be admitted to hospital.

There is one palliative care room at the Centre. At the present time, there is no scientific method to identify the frequency of use and the number of palliative care days generated by palliative care patients at the Centre. South Eastman is presently in the process of creating a data bank which will provide some of that information. For the purpose of this review, the inpatient statistical report clearly identifies the number of deaths for 2003, 2004 and the 1st six months of 2005 to be 20, 21 and 26 respectively⁹. According to the Palliative care coordinator, there are at any given time in the region 40 to 50 palliative care patients enrolled in the palliative care program and there are approximately 80 palliative care deaths each year in the region.

As identified in the community health assessment 2003/04, South Eastman does not have a hospice component to its palliative care program. A hospice is

“A facility where specially trained doctors, nurses and others care specifically for people who are dying. Care may be provided throughout the course of the terminal illness or be intermittent to provide respite for care givers or to have difficult symptoms brought under control and improve quality of life”.

Considerations for the future of Acute Care services at DeSalaberry Health Centre

Population growth and acute care hospitals:

- the overall rates of hospitalization in the region have declined markedly since 1999/2000 despite a steady population growth;
- persons age 65+ are by far the highest users of hospital services, rates of acute hospitalisation and day use rise dramatically with advancing age;
- in the South Eastman population, residents age 65+ account for nearly three quarters of hospital days used in South Eastman;
- seniors age 75+ are especially vulnerable to hospitalization; even though they represent only 5% of the South Eastman population, they account for approximately 60% of hospital days used in the region.

⁹ It is interesting to note that there has been no entry for palliative care days in Saint-Pierre in the Regional QI stat summary for 2004/05 given the number of deaths recorded. For the first 6 months of 2005/06, 24 palliative care days has been reported.

Bed utilisation:

- the low utilisation of hospital beds at The Centre is problematic;
- 25% of all inpatient days are being generated by individuals who should not be in an acute care bed;
- there will be an ongoing need for alternate care beds at The Centre as a result of the waiting list at the Repos Jolys;
- ineffective use of acute care resources.

Palliative care:

- as the South Eastman population increases and ages, the number of people living with cancer and other life-threatening illnesses, the need for access to quality, flexible and individualized cancer care and palliative care services are escalating;
- a concrete plan has yet to be developed by the region related to access to palliative care/hospice care.

South Eastman strategic plan (2006-2011):

Strategic priority related to Acute Care is: “to work towards equitable in-region access to quality care services for all South Eastman residents”. Other than indicating that “core acute care services are provided by four hospitals, including ER” there is no specific direction in the strategic plan to address the appropriate utilisation of acute care resources at The Centre.

Strategic priority related to palliative care: “to enhance quality of living for residents reaching the end of life” by “increasing access by clients and their families to end-of-life care delivered according to palliative care principles and by strengthening the regional palliative care program”.

Recommendations for the hospital – Acute care services

To ensure viability of the hospital - Acute Care, it is recommended that:

- **there be a designated number of acute care beds to accommodate community needs;**
- **there be a designated number of “alternate care beds” for individuals awaiting panelling, placement or home care;**
- *** the Centre house a regional program such as hospice care – concept of a centre of excellence, which could provide services both in French and English (other regional program such as rehabilitation services, alternate care program or respite care could also be considered);**

- **provide an educational program (to coincide with the regional program) and develop an ongoing continuing education program to ensure the provision of quality services;**
- **workload and staffing patterns be reviewed to accommodate patient mix and occupancy;**
- **review the feasibility of sharing of the LPN between the acute care and the Repos Jolys on the evening and night shifts and on the weekends.**

*** Note: More community consultations and discussions with South Eastman need to occur in order to identify the most appropriate regional program to be housed at The Centre.**

Hospital - Emergency room services (ER)

Four physician positions are funded by the regional health authority to provide health services at the Saint-Pierre medical clinic, including providing services to the hospital, the ER and the personal care home. The provision of on-call services for the ER is shared between the four physicians in Saint-Pierre and most recently a fifth physician from Niverville has been added to the rotation. The sharing of the on-call amongst the five physicians means that a physician is on 24 hour call one day in five.

Up until September 2005, the hospital's emergency room (ER) was opened 24 hours a day 7 days per week. The recent departure of two of the physicians resulted in the ER being closed 7 days/week between the hours of 2000 to 0800. As of December 12, 2005, there are now three practicing physicians in Saint-Pierre, a fourth is presently being recruited. The on-call rotation is now being shared between 4 physicians resulting in a 1 in 4 on-call schedule.

Five (5) levels of care are used in the ER to categorize patients. The levels of care are based upon the urgency of the problem. The level of care (triage) is determined using a set of approved medical guidelines and is consistent throughout the province of Manitoba. The patient will receive an assessment and treatment according to the level of care as follows:

- Level 1: Patient needs **resuscitation**, patient will be seen by a nurse and as soon as possible by a physician (heart attacks).
- Level 2: Refers to an **emergent visit**, patient will be seen by a nurse and as soon as possible by a physician, includes:
 - major trauma, severe head injury, amputation, severe difficulty breathing, anytime a person is unconscious, severe bleeding.
- Level 3: Refers to an **urgent visit**, the patient will be seen by a nurse within 30 minutes, includes:
 - head injury but still awake, chest pain that is possibly a heart attack, signs of serious infection, large or deep cuts, broken bones or injuries which threaten limb.
- Level 4: Refers to a **less urgent visit**, the patient will be seen by a nurse within 1 hour, includes
 - possible broken bones or sprains, back pain, skin/wound infections, migraine headaches, chest pain not resembling a heart problem.
- Level 5: Refers to a **non urgent visit**, the patient will be seen by a nurse within 2 hours and includes such things as:
 - a cold or sore throat, ear ache, requests to refill prescription.
- Scheduled visit: Refers to a visit that is **scheduled**, includes:
 - procedures such as casting and removal of stitches.

A few years ago, nurse managed care was introduced to the emergency department as a pilot project. Nurse-managed care is medical care provided by a registered nurse (RN). Special training was provided to the RNs by the Faculty of Nursing at the University of Manitoba. RNs are qualified to provide care to patients whose level of care has been rated at Level 5. This service ensures that the ER is available to provide treatment to life threatening or serious illness or injury.

In the last 4 fiscal years, the volume of clients being seen in the ER remains fairly stable with an average of 3,323 visits per year. This number translates to an average of 9 to 10 visits per day. An analysis of the statistical data from the ER (Table 2) reveals that between 85 to 90% of all visits consist of scheduled non urgent and less urgent visits. These are visits that do not require to be seen in the emergency department. The 2004 community survey on medical services, the staff interview and the recent staff survey indicate that clients are using the ER for non emergencies because they are unable to access physicians in the medical clinic.

Table 2: Number and types of emergency room visits for fiscal years 2001/02 to 2004/05.

Fiscal year	Total visits	Resuscitation % & (# of visits)	Emergent % & (# of visits)	Urgent % & (# of visits)	Less Urgent % & (# of visits)	Non-urgent % & (# of visits)	Scheduled % & (# of visits)
01/02	3481	.3 (9)	2.1 (73)	12.2 (426)	26.6 (925)	43.3 (1506)	15.6 (542)
02/03	3140	.2 (6)	2.1 (66)	11.4 (359)	45.1 (1416)	21.7 (681)	19.5 (612)
03/04	3243	.21 (7)	2.1 (68)	9.9 (322)	43.7 (1417)	25.8 (837)	18.3 (592)
04/05	3429	.17 (6)	1.4 (47)	8.3 (285)	41 (1405)	22 (754)	27.2 (932)

If an alternate strategy was in place for these outpatient visits the average number of true emergencies in the last 4 fiscal year would be approximately 420 visits per year which would translate into just over one (1) visit per day.

It is also interesting to note that in the Saint-Pierre medical clinic management review, the data revealed that by far the majority of visits occur during the day shift. ER stats were analysed over a 6 month period from January to June 2004. The data revealed that 61.7% of ER visits occurred during the day shift (0800 - 1600), 32.6% occurred on the evening shift (1600 – 2400 hrs) and only 5.7% of visits occurred on the night shift (2400 to 0800) hours. If that same distribution was applied to the average number of true emergency visits (420) in the last four fiscal years, this would represent an average of 259 visits during the day shift, 137 during the evening shift and only 24 visits during the night shift.

A number of negative effects are being experienced by both physicians and nurses related to the number of non emergency visits to the ER:

- Heavier workload being generated for physicians when on-call: during the day shift this takes physicians away from the clinic and on the evening and night shifts, it impacts on their quality of life and thereby affecting physician retention.

- The only RN on all shifts (minimum staffing levels) must triage all the non urgent visits (an average of 950 visits/year), as well as provide care to all other visits to the ER. In addition the RN must provide care to inpatients and residents of the Repos Jolys when required.
- A low volume of true emergencies prevents existing staff from maintaining and improving their skills.
- Instability in the service delivery i.e. inconsistencies in the quality of care provided, prolonged wait times, intermittent closures of the ER.

Considerations for the future of ER services at DeSalaberry Health Centre

ER utilisation and service provision:

- physician recruitment and retention;
- low volumes of true emergency visits;
- high volumes of non emergency visits;
- maintenance and improvement of skills;
- quality of services provided;
- effectiveness of nurse managed care;
- workload considerations (minimum staffed facility) and on-call rotations;
- instability in service delivery.

South Eastman strategic plan (2006-2011):

There is no mention of The Centre's ER in the region's strategic plan other than to indicate that the ER service will continue as part of the core acute care services. There is however, an emphasis on "enhancing the ER departments at the Bethesda and Ste. Anne hospitals, with pursuit of full time ER physicians and staff to meet the needs of the increasing numbers of patients".

Recommendations for the Emergency Room

It is recommended that a strategy be developed and implemented over the next five (5) years to enhance ER services at The Centre and increase the number of true emergent visits to ER which includes:

- **develop a communication and community education strategy re:**
 - **status of ER services;**
 - **use of ER services and community mobilization;**
 - **5 year plan;**
- **that ER services continue to be offered 24/7;**

- that the less urgent, non-urgent and scheduled visits be seen in an alternate setting (primary health care centre);
- that EMS be further enhanced (paramedic trained staff to improve pre hospital emergency care) to support the ER services;
- that the use of ER services be monitored yearly and evaluated after year 5 to determine viability of ER services at the Centre;
- that renovations of the operating room (OR) theatre into an additional emergency room be placed on hold until viability of ER services is determined;
- review the concept of nurse managed care in ER and its effectiveness;
- provide ongoing staff education to ensure the provision of quality services.

Note: This recommendation is contingent on being able to recruit and retain physicians.

Hospital - Laboratory and diagnostic services

Laboratory and diagnostic services in South Eastman is not managed by the region. This function is assumed by the Diagnostic Services of Manitoba (DSM) Inc. DSM manages all public laboratories in the province and rural diagnostic services and as such, the Regional Diagnostic Manager in South Eastman reports directly to DSM. DSM's strategy is to be a quality leader in laboratory medicine, diagnostic imaging and pathology in Canada and maintain or enhance existing standards¹⁰.

The range of diagnostic services available at The Centre assists physicians in monitoring health and diagnosing illnesses. Diagnostic services being provided at the Centre include cardiology (electrocardiogram (ECG) and blood pressure monitoring), x-ray and laboratory services (haematology - complete blood counts, cardiac tests, etc. and chemistry testing).

One (1) FTE position is funded in the X-ray department. The services are provided by two part time staff. Recently, the hours of operation for the department have been increased to 8 hours per day (0830 to 1630) five days per week. An on-call service is provided outside of the hours of operation.

Laboratory services were provided by 1.0 FTE i.e. a laboratory technologist. Recently a .4 FTE phlebotomy/clerical staff was added to cope with the increased workload. The hours of operation for the department is from 0830 to 1630 five days a week and 0900 to 1200 hours Saturday mornings. Once a month for ½ day, the laboratory technologist provides services to Menno Home in Grunthal.

The laboratory staff provides limited on-call services. In the past, standby services were provided on a volunteer basis. Since the departure of those staff, the present staff has volunteered for limited standby availability (until 2200 hours weekdays). To maintain access to after hour services, a local volunteer transport system as well as the Steinbach taxi is available to transport samples to Steinbach or Winnipeg as necessary for processing. Training has been provided to nursing staff and health care aides (HCA) for the collection of specimens during the off hours.

To maintain quality services within the diagnostic departments, the regional health authority has purchased new equipment (haematology analyzer, blood bank fridge, coagulation analyzer, X-ray cassettes, chemistry analyser, etc), added additional testing capability and training for staff (cardiac marker tests), increased staffing (.4 FTE phlebotomy/clerical staff) and has developed policies and procedures to meet provincial requirements.

¹⁰ Diagnostic Services of Manitoba Inc. (September 2005). www.dsmanitoba.ca

Table 3: Statistical data for laboratory and diagnostic services including number of diagnostic tests, workload units and call backs.

Diagnostic Tests	2002/03	2003/04	2004/05	*2005/06
Laboratory	33,478	34,763	33,740	15,924
ECG monitoring	810	757	523	264
X-ray	2,135	1,843	1694	1071

Workload Units	2002/03	2003/04	2004/05	*2005/06
Laboratory	184,017	198,988	193,090	91,167
ECG monitoring	12,718	15,367	10,672	5,211
X-ray	21,110	18,746	16,900	10,369

Call-backs	2001/02	2002/03	2003/04	*2004/05
Laboratory	170	159	115	82
X-ray	242	343	209	215

* Stats are for the first 6 months of the fiscal year only.

The number of laboratory tests and workload units has remained fairly stable in the last 3 fiscal years. There is a slight decrease in the number of EKG's performed and a 20% decrease in diagnostic testing in the X-ray department between fiscal years 2002/03 and 2004/05 with a corresponding decrease in workload.

Considerations for the future of Laboratory & Diagnostic services at DeSalaberry Health Centre

Service requirements:

- improving access;
- maximizing staffing resources;
- ensuring quality services.

South Eastman strategic plan (2006-2011): There is no specific strategic priority identified in the health plan relating to laboratory and diagnostic services.

Recommendations for Laboratory and Diagnostic services

In order to enhance the quality of and the accessibility of services, it is recommended that:

- **laboratory and radiology staff be crossed trained as soon as possible to improve access to services (including on call services) to the ER, acute care and medical clinic;**

- **administrative and support workload (e.g. completion of forms, paper work, etc.) in the laboratory and X-ray department be reviewed to identify potential for sharing of workload amongst existing staff resources;**
- **hours of work for laboratory and diagnostics be reviewed to coincide with the extended hours which could be provided in a primary health care centre;**
- **laboratory services be offered at the Chalet Malouin a half day per month;**
- **provide ongoing staff education to ensure the provision of quality services;**
- **quality monitoring be a priority in both laboratory and diagnostic services to ensure quality of services.**

5.1.5 Personal Care Home (Repos Jolys)

The Repos Jolys is a 22 bed bilingual personal care home (PCH) facility. It is always utilized at capacity. The average wait list is between 15 to 18 with 17 individuals who are presently awaiting admission to the Repos. On average, individuals stay on the wait list for approximately one year from the time of panel. Individuals awaiting placement can often be found occupying an acute care bed at The Centre.

The average age of the residents is between 85 to 88 years old. Residents are categorized according to the level of care required. Levels of care are described as:

- | | |
|----------|--|
| Level 1 | Residents are relatively independent; |
| Level 2: | Residents experiencing mild confusion and require minimum assistance; |
| Level 3: | Residents requiring significant personal care and ongoing supervision, they are categorized as “partial dependence”; |
| Level 4: | Residents requiring significant personal care and ongoing supervision, they are categorized as “maximum dependence”. |

Only 2 residents of the Repos Jolys are categorized in level 2, 10 residents are in level 3 and 10 in level 4. The care requirements are thus considerable.

Palliative care services are also provided at the Repos Jolys when required. Residents are not transferred to the “hospital” to receive palliative care.

Considerations for the future of the Repos Jolys

Level of care requirements:

- heavy workload for staff;
- appropriate staffing level;
- ensuring quality services.

Recommendation for the Repos Jolys

As a result of the increasing workload, it is recommended that:

- **the allocation of clinic and administrative responsibilities of the Clinical Resource Nurse be reviewed;**
- **staffing be reviewed to ensure consistency with the long term care staffing guidelines;**
- **review the feasibility of sharing of the LPN between the acute care and the Repos Jolys on the evening and night shifts and on the weekends;**
- **provide ongoing staff education to ensure the provision of quality services.**

6.0 Staff survey – Comments

It is important to underline that many administrative changes were occurring at The Centre while the staff survey was being completed. It appears that many of the issues and concerns that were brought to the forefront by the staff are presently being addressed. Among some of the changes that have occurred are:

- the appointment of a new facility manager for The Centre;
- the new hire of a .8 FTE CRN for the acute care facility;
- review of HCA staffing for the acute care facility;
- two full time staff development positions in the region, one for long term care and one for acute care.

These changes in and of themselves however, will not improve the work environment unless a clear strategy is established to deal with the staff issues. A few suggestions are brought forward for the consideration of South Eastman senior staff, recognizing that work has already begun on establishing a plan of action:

1. Identify and implement strategies to manage change – this will go a long way in creating an environment that is supportive of staff, residents, families and the community. Staff has provided good suggestions.
2. Review and perhaps revise the organisational structure to reflect clear lines of accountability; one could consider a direct reporting relationship between the Regional Manager position and the Facility Manager.
3. Staff has made good suggestions to improve the quality of services, it would be important to revisit these in detail.
4. Involve staff in developing an action plan to ensure the future of stable and sustainable health services at The Centre.

7.0 Community participation

What are the next steps for the community?

This report was intended to inform and provide direction to the community for future planning of stable and sustainable health services at The Centre. Further steps will need to be pursued if the proposed recommendations in this report are to be implemented; these can include:

1. **Community consultations:** The community – the RRHC, the Coalition, and other key stakeholders will need to be consulted to either accept or refute the proposed recommendations. A shared vision and the commitment to move forward by all community groups will be essential to the successful outcome of this strategy.
2. **Share report:** The report will have to be distributed to key stakeholders i.e. South Eastman Senior Staff and Board of Directors, staff of The Centre, physicians, etc.
3. **Leaders/champions:** A core group of a few community individuals will have to be identified to become the leaders or the champions of this strategy.
4. **Meeting with South Eastman Health:** An initial meeting should be held between the “core group” and Senior Staff of the region to review and discuss the report. Further meetings will need to occur once the Board of Directors of South Eastman Health has given its approval. At this stage, it will be important to clearly articulate the role of each of the key stakeholders in the process (if the process is to continue), including how the community will continue its participation. Key questions for the community will be:
 - a. What will the community want to achieve through their ongoing participation?
 - b. What level of participation is likely to be appropriate and acceptable?
5. **Provision of bilingual services:** Since the primary health care centre will have a unique focus i.e. the provision of French services to the francophone communities, this project should be brought forward to the Conseil Communauté en Santé to ensure its inclusion in the provincial plan for the provision of French language services (Setting the stage/Préparer le terrain). Special funding may be available from the Federal government for this initiative.

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Appendix A

Staff Survey

Centre médico social DeSalaberry Health District (The Centre)

Staff survey

(Please return survey on or before **Friday November 4, 2005**. If you require additional space for your comments, simply insert another page).

The Centre and its operations

1. What do you appreciate the most at the Centre?

2. What do you appreciate the least at the Centre?

3. Given the opportunity what would you change?

4. How can we improve the operations at the Centre?

Client/patient/customer service

1. How would you rate the quality of services being provided at the Centre?
☐ poor ☐ good ☐ very good ☐ excellent
2. Comments

3. How can we improve the quality of services?

Staff needs

1. How would you describe the quality of your worklife?

☐ poor ☐ good ☐ very good ☐ excellent

Comments:

2. How can your employer be of support to you?

Programmes and services

1. What is your perception of the role of the Centre in the South Eastman Health Region?
2. What is your perception of the **future role** of the Centre in the South Eastman Health Region?
3. From your perspective, are there other programmes and/or services that should be offered at the Centre? Which ones? Why?

Other comments:

Name and classification (optional):

If you would prefer to have an interview please leave your contact information and Suzanne Nicolas will contact you to set up an interview date and time.

Appendix B

What is primary health care?

What is primary health care?

It is important to acknowledge that the majority of the factors that influence the health of the population fall outside of the realm of the health system. Research indicates that health services contribute only about 25% to the health status of the populations. Genetics and biology account for another 15% and the physical environment for 10%. A full 50% of factors influencing population health fall within the scope of social and economic environments¹¹.

Primary health care (PHC) refers to an approach to health and describes a variety of services beyond the traditional health care system¹². It describes the services that we receive at the first level of contact with the health system – e.g. with a physician's office, a health clinic, a pharmacy, or a community health centre¹³. The first level of contact can also be outside of the traditional health system for example, through community organisations, church groups, the educational system, the justice system, etc. PHC includes.

- primary care, which is the component that focuses on health care services, including health promotion, illness and injury prevention and the diagnosis and treatment of illness and injury; primary care services for the most part are delivered by family physicians;
- all services that play a part in health such as income, housing, education, and environment.

Organization of primary health care

In 2001 the Canadian Health Services Research Foundation (CHSRF) commissioned a team of researchers to explore the organization and delivery of primary health care models in Canada and around the world¹⁴. The researchers analysed 28 cases of primary healthcare organization in the industrialized countries, inventoried 250 community health centres, examined all the relevant studies on the topic (38 studies) and consulted 13 experts in the field of primary health care.

¹¹ South Eastman Health/Santé Sud-Est Inc. (2003/04). Community Health Assessment: Comprehensive report.

¹² Health Canada (November 2005). Health Care System: About primary health care.

¹³ Canadian Institute for Health Information (2003). Health care in Canada.

¹⁴ Canadian Services Research Foundation (November 2003). Choices for change: The path for restructuring primary healthcare services in Canada.

A brief overview of key findings from the research demonstrated that:

- no one organizational model for delivering primary health care can meet all the anticipated effects i.e. effectiveness, quality, access, continuity, productivity and responsiveness;
- 4 predominant models of PHC are identified:
 - 2 community models
 - integrated community model
 - non integrated community model
 - 2 professional models
 - professional contact model
 - professional co-ordination model
- 2 models of PHC organization and delivery are favoured – the integrated community model and the professional coordination model;
- the combination of the integrated community model and the professional coordination model maximizes all desired effects in the delivery of PHC.

Overview of the models of PHC

Community primary health care models:

The community approach to PHC is designed to improve the health of communities living in specific geographic areas and to promote the development of the communities served. Its mandate is to meet the healthcare needs of the population and to provide it with all the medical, health, social and community services required.

The community approach is divided into 2 models: the integrated community model and the non-integrated community model. The difference between the two models is the degree to which they are integrated into the rest of the healthcare system.

The **integrated community model** promotes integration of primary healthcare into the other components of the healthcare system, including:

- use of information technology to convey clinical information within the healthcare centres and to other service providers serving the same population for example, private practice, hospitals;
- responsibility for continuity of care is assumed by the care team;
- availability of services 24/7;
- co-operation with other health care providers.

This model focuses on co-operation and interaction with the community.

The **non-integrated community model** lacks specific integration mechanism into the rest of the health care system. It has the following characteristics:

- it uses no information technology or other ways to integrate services with those provided by the rest of the health care system;
- services are not available 24/7;
- no formal mechanism ensures continuity of services.

This model offers as broad a range of services as that of the integrated community model but the health centre provide these services directly, with no collaboration with other parts of the health care system.

Professional primary healthcare models:

Professional models of primary healthcare are designed to deliver services to patients who seek their services or to individuals who choose to register with a specific physician. Usually the responsibility falls to physicians working alone or in small groups. The public plays no role in these organisations. The health care team consists of physicians with whom nurses may also be associated. The range of services provided is limited to preventive, diagnostic or curative medical services.

There are two types of professional models: the contact model and the coordination model:

In the **professional contact model** services are provided almost exclusively by physicians practising alone or in groups. Physicians are rarely associated with other health care professionals and there are no formal mechanisms for integration of services into the other part of the health system.

The purpose of the **professional coordination model** is to provide continuous services over time to patients who register with the organization. The health care team consists of physicians and nurses and one of the professional staff (usually the physician) is designated to provide follow-up and continuity of care. Information technology is used to transfer clinical information to other providers in the system. As well, a nurse is a liaison with other components of the system and coordinates clinical integration. This approach guarantees continuity and good coordination of services.

Appendix C

Primary health care model for The Centre

Primary health care model for The Centre

On the basis of an extensive literature review and the above noted research by the CHSRF, the author has proposed a model for the organisation of PHC for The Centre (Figure 1). To ensure a fully integrated service delivery model, the providers of services within the various components of the model must develop a collaborative working relationship (établir une interdépendance entre les parties).

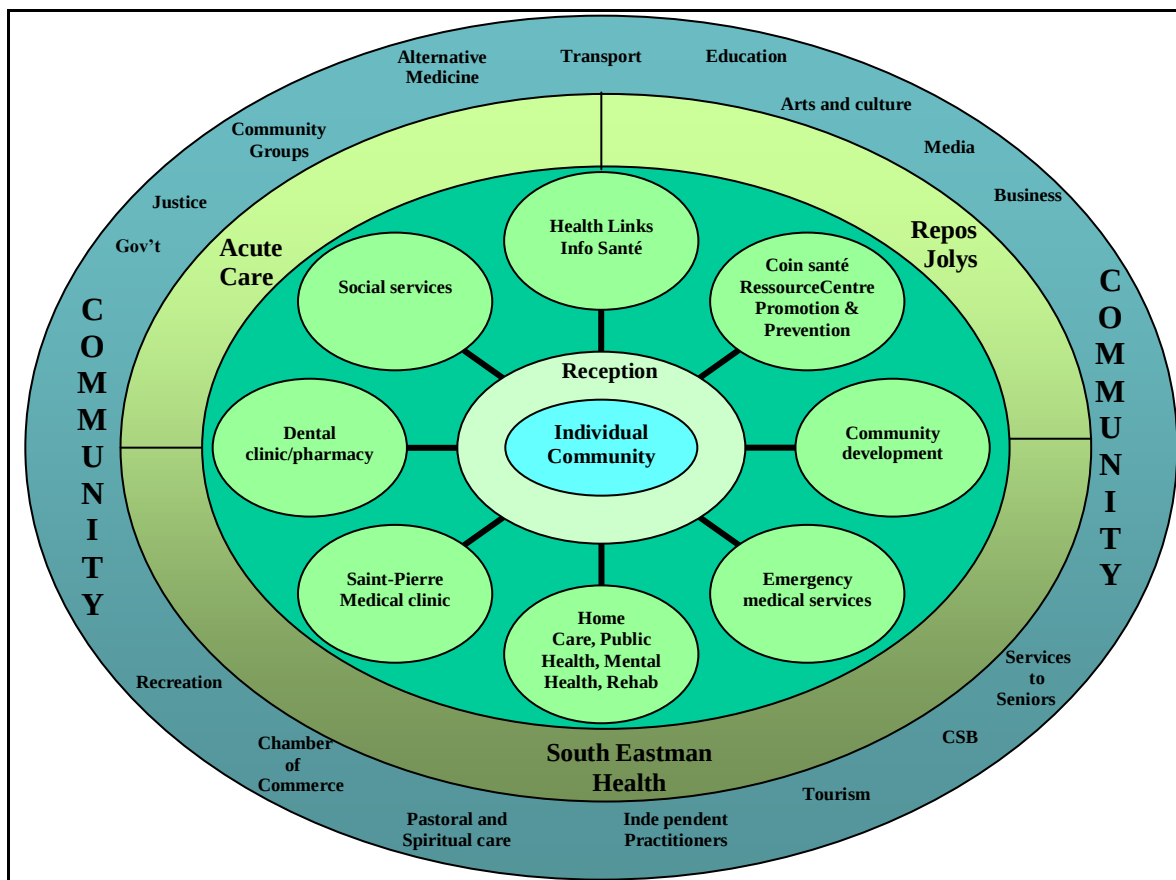
The underlying principle of the model is that primary health care providers are active throughout the continuum of health services by promoting health, preventing disease, managing chronic disease and caring for those who have minor illnesses or injuries.

Consistent with the four pillars of PHC ¹⁵, the PHC team will:

- work together as a **team** to improve care and reduce duplication,
 - will acknowledge the individual, the family as part of the team;
- **share clinical information** between health care providers to improve quality, access and coordination of health services
 - use information technology to promote sharing of information;
- provide **access** to the right care at the right time, including advice, information and care outside of regular office hours
 - coordinate and integrate all existing primary health care services at The Centre;
 - facilitate and coordinate access to acute care, ER, long term care, and other health programs and services in South Eastman;
 - develop intersectoral partnerships to improve the health of the community (i.e. Chalet Malouin, Centre de services bilingues/Bilingual Centre, Services to seniors, the local schools, justice system, economic development, the private sector, etc.).
- focus on **healthy living**
 - prevention of illness and injury, that promotes self care and manages chronic conditions

¹⁵ Health Canada (2004). Primary health care website : www.primaryhealthcare.ca

Figure 1: Primary health care model for The Centre



Description of the components of the primary health care model:

The description of the PHC services that follow is not to be interpreted as being all inclusive. A host of other services could be incorporated within a PHC centre. These represent some of the services presently being offered at The Centre.

The community

The community is at the centre of this model. The community is empowered to assume responsibility for its health and well-being. In so doing, the individual, the family, community groups, become active members of the health care team. The model acknowledges the necessity for individuals and populations to access the continuum of health services in order to deal effectively with the health problems of a population.

Reception services (may be centralized or decentralised for the various components)

- provided on site or on telephone
- first contact with the PHC
- reception staff :
 - provide information on services offered at the PHC

- direct the client to the most appropriate service within the PHC
- make appointments for clients
- ensure a valid MHSC card
- can refer client to appropriate service in the community

Health Links/Info Santé

- telephone health information line, offers services 24/7 in many different languages
- provide health advice, information
- assessment of client and determine urgency of situation
- can direct to the most appropriate services for follow up care
- referral to ER or other health services if required

Coin Santé/Health Corner:

- program planning for health promotion and disease and injury prevention
 - close links with the community facilitator, other health care professionals and the health system (acute care, Repos Jolys, community services etc);
- provides the programs and services in the community
 - schools
 - day care centres
 - target groups (seniors, women, men, immigrant, families, etc.)
 - community groups
- establishes links with other existing community programs
 - services to seniors (Manoir & Chalet Malouin)
- organises support groups
- a video and book lending service in conjunction with the Regional library
- access to the l'Internet
- distribution of educational pamphlets, brochures

Community developement :

- staff is the liaison between the community and the health system, is responsible for :
 - ongoing community health assessments
 - build community capacity and assists in community mobilisation
 - is a resource person and provides support to community organisations
 - works with the community to address initiatives related to the determinants of health

Primary care:

- these refer to services which are primarily provided by physicians and at times by nurses

- services can be provided by appointment and/or walk-in, includes:
 - assessment, diagnostic, curative
 - health promotion, prevention
 - tele-health

Emergency medical services

- integration of EMS staff in the continuum of health services delivery
 - primary health care (stabilization in the home & transfer, education, health promotion, prevention)
 - acute care/ER

Community services

- Coordination and integration of all community services:
 - Home Care
 - Public Health
 - Mental Health
 - Midwifery

Although specific services are not managed by South Eastman Health, these services are an integral component of a PHC centre. Efforts to include these health providers in a strategy to improve population health are crucial. Some of those services could include:

Social services

- Includes :
 - family services and housing
 - child and family services

Dental clinic/Pharmacy services

Others